

STATION 01: A stem written in a paper with an HIV patient with a low CD4 count, then the use of OPTHALMOSCOPE, they assess how to do fundoscopy systematically, and the possible findings you may see

STATION 02 case scenario written in a paper with pulmonary function tests parameters,(a).To calculate the difference in the FEV/FVC ratio after the given bronchodilator? (b). What is the diagnosis?=ASTHMA

(c) What are the medications used to relieve the symptoms of acute exacerbation in these scenarios?

STATION03: (a). name device = meter dose inhaler=MDI

(b) How to use a meter dose inhaler-[HOW TO USE MDI](https://lungfoundation.com.au/support-resources/resource-hub/pressurised-metered-dose-inhaler-pmdi-puffer-inhaler-device-technique-fact-sheet/)

<https://lungfoundation.com.au/support-resources/resource-hub/pressurised-metered-dose-inhaler-pmdi-puffer-inhaler-device-technique-fact-sheet/> (c). What are the other types(3)of the device above?-DPI=dry powder inhaler-SMI=Soft mist inhaler

STATION04:Patient with parkinson disease: (a).do motor system exam

(b).Dx.(c).DDx.(d). Mechanisms of disease pathophysiology

STATION05:Patient with rheumatic heart disease: (a). Do a Precordial examination.

(b)What are the finding=pansystolic murmur for Rheumatic heart disease (c). What is the differential diagnosis? (d) What are the complications

a. Precordial Examination

A precordial examination involves a systematic assessment of the chest area over the heart to evaluate its size, rhythm, and sounds. The steps are:

1. **Inspection:** Look for visible pulsations, heaves, or thrills.
2. **Palpation:** Feel for the location, size, and character of the apex beat. A displaced apex beat can indicate an enlarged heart. Also, feel for any heaves (lifting of the chest wall with each heartbeat) or thrills (a palpable murmur).
3. **Auscultation:** Listen to the heart sounds using a stethoscope at the five main areas: aortic, pulmonary, tricuspid, mitral, and Erb's point.

b. Finding: Pansystolic Murmur for Rheumatic Heart Disease

A **pansystolic murmur** is a heart murmur that begins with the first heart sound (S1) and continues throughout the entire systole, ending at the second heart sound (S2). In the context of rheumatic heart disease, a pansystolic murmur is typically caused by **mitral regurgitation**. This happens when the mitral valve is damaged and fails to close properly, causing blood to flow backward from the left ventricle into the left atrium during systole.

c. Differential Diagnosis and Complications

Differential Diagnosis for a Pansystolic Murmur

While mitral regurgitation is a common cause, other conditions can also produce a pansystolic murmur:

- **Tricuspid regurgitation:** Backflow of blood from the right ventricle to the right atrium.
- **Ventricular septal defect (VSD):** A hole in the wall separating the ventricles.

Complications of Rheumatic Heart Disease (RHD)

RHD can lead to serious long-term complications, even after the initial rheumatic fever episode has resolved. These complications often arise from the progressive damage to the heart valves.

- **Heart failure:** The heart muscle can become overworked and fail due to the constant strain of damaged valves.

- **Atrial fibrillation:** The left atrial enlargement caused by mitral valve disease can lead to an irregular and rapid heart rhythm.
- **Stroke:** In patients with atrial fibrillation, blood clots can form in the atria. If a clot breaks off and travels to the brain, it can cause a stroke.
- **Infective endocarditis:** The damaged heart valves are more susceptible to bacterial infection.
- **Pulmonary hypertension:** Increased pressure in the left atrium can back up into the lungs, leading to high blood pressure in the pulmonary arteries

STATION 06: Patient with ascites: (a).do inspection and percussion (b).Differential diagnosis (c). How will you evaluate ascitic fluid

STATION 07: Patient with macules in the face and a hand nodule

a. What is Dx-SLE? What is DDx? What are the complications

STATION 08: There is a dialyser tool kept

a. name of device (b) indication of the tool (c) complications of using the tool

a. Name of the device

The device is a **dialyzer**, also known as an artificial kidney. It is the component of a hemodialysis machine that filters the blood.

b. Indication for the tool

The dialyzer is indicated for patients with **end-stage renal disease (ESRD)** or **acute kidney injury (AKI)**. Its primary function is to perform the tasks of a healthy kidney, which include:

- Removing excess fluid from the blood.
- Removing waste products, such as urea and creatinine, from the blood.
- Correcting electrolyte imbalances.

c. Complications of using the tool

The use of a dialyzer and the hemodialysis procedure can lead to several complications:

- **Hypotension:** A drop in blood pressure, which is the most common complication.
- **Muscle cramps:** Often caused by the rapid removal of fluid and electrolytes.
- **Bleeding:** Can occur at the access site or as a result of the heparin used to prevent clotting in the dialyzer.
- **Infection:** The access site (e.g., fistula, graft, or catheter) is a potential entry point for bacteria.
- **Anaphylactic reaction:** A rare allergic reaction to the dialyzer membrane itself.

STATION 9: ECG with Atrial Fibrillation

a. What is the diagnosis? (b) What are supportive ECG features? (c) 2 scores used in the diagnosis? (d) What are 5 causes of diagnosis

A. Diagnosis

The diagnosis is **Atrial Fibrillation (AFib)**. It is a type of arrhythmia characterized by rapid, irregular electrical activity in the atria of the heart.

B. Supportive ECG Features

- **Irregularly irregular R-R intervals:** The time between each QRS complex is inconsistent and random.

- **Absence of P waves:** The normal, distinct P waves, which represent atrial depolarization, are not visible.
- **Fibrillatory (f) waves:** Instead of P waves, the baseline may show fine, irregular oscillations or fibrillatory waves.
- **Narrow QRS complex:** The QRS complex is usually narrow (<0.12 seconds), as ventricular depolarization is normal, though it may be wide if there is a pre-existing bundle branch block or accessory pathway.
- **Ventricular rate > 100 beats/min:** The ventricular rate is typically fast in new-onset AFib.

C. Scores Used in Diagnosis

Two important scores used in the management and diagnosis of AFib are:

- **CHA₂DS₂-VASc Score:** This score assesses the risk of a patient with AFib having an embolic stroke. It is used to determine the need for anticoagulation. The score assigns points for various risk factors, including Congestive heart failure, Hypertension, Age >75 , Diabetes, Stroke/TIA, Vascular disease, Age 65-74, and Sex (female).
- **HAS-BLED Score:** This score assesses the risk of a major bleeding event in a patient with AFib who is on anticoagulation. It is used to weigh the benefits of anticoagulation against the risks. The score considers risk factors like Hypertension, Abnormal liver/kidney function, Stroke, Bleeding history, Labile INR, Elderly (>65), and Drugs/alcohol use.

D. Five Causes of Atrial Fibrillation

1. **Hypertensive heart disease:** High blood pressure is a major risk factor and a common cause.
2. **Valvular heart disease:** Especially mitral stenosis and mitral regurgitation, which can cause atrial enlargement.
3. **Coronary artery disease:** Ischemia can irritate the atria and trigger AFib.
4. **Hyperthyroidism:** An overactive thyroid gland increases heart rate and the risk of AFib.
5. **Alcohol use:** Binge drinking ("holiday heart syndrome") or chronic alcohol consumption can precipitate AFib.

OSCE IM GROUP F

1. 24y/o with a chancre at his penile shaft and later presented with multiple lesions on his hands



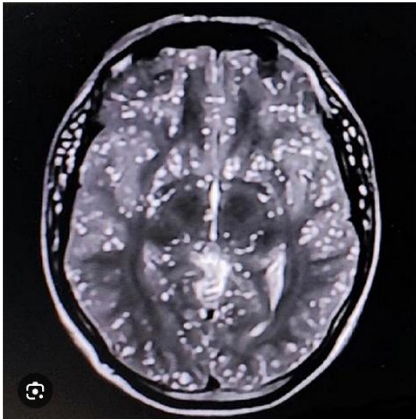
- a. Describe the skin lesions
 - b. What is the diagnosis
 - c. what are your differentials
 - d. what is the confirmatory investigation
 - e. what is the initial treatment
2. Diabetic pt not on antiglyemics, has cough with green sputum for 6 days; presents to EMD with confusion. RBG is high, has hypotension has sweet breath on examination
- a. What is the diagnosis for the 6 days presentation?
 - b. what other 4 tests other than RBG
 - c. mention 2 likely causes of confusion in this patient
 - d. how to manage the patient
3. Immunocompromised Pt with neck stiffness, headache, and blurred vision. Performed CSF with following results.....
- CSF appearance -Clear*
Lymphocytes -85%
Protein -1.8g
Csf Glucose - 2.5
Serum Glucose -5.5
Opening pressure -18cm of H2O Interpret the CSF results
- a. what is the diagnosis
 - b. what are your differentials
 - c. what other tests would you perform for CSF
 - d. how to manage the patient

4. Pt with foul smelling productive sputum; drenching night sweats, fever.



Given a chest xray...

- Interpret the findings in CXR
 - what is the diagnosis
 - possible causative organism
5. 16y/o female from urban mountainous region. Presenting with altered mental status, sided weakness. MRI of the brain showed...



- Interpret the findings in the MRI
 - What is the diagnosis
 - what other test would you perform to confirm the diagnosis
 - how to manage/treatment
6. Pt with shortness of breath given inhaled corticosteroid. Spirometry done before and after bronchodilators, results were....
- Table ya result ilikuwa inaonesha FEV1 before broncodilators and after plus ilikuwa na predicted value*
 - Calculate the change in FEV1
 - interpret the results

- d. what is your diagnosis
 - e. ATS/ERS criteria
7. 54y/o presents with abdominal distension and yellowish discoloration of the eyes. Reports to heavily drink Konyagi
- a. What is your diagnosis
 - b. what are your expected signs you will see on abdominal examination
 - c. what is your differential diagnosis
 - d. what other investigations would you perform
8. CBC results showed....
- a. *The CBC had Low Hb but High WBC,PLT plus hata monocytes, eosinophils and basophil were kinda elevated. Plus kulikuwa na myelocytes na metamyelocytes.*
 - b. What is your diagnosis
 - c. what are your differentials
 - d. what are other investigations would you do to confirm your diagnosis
9. Pt presented with PND, orthopnea requiring 4 pillows, shortness of breath while climbing a flight or stays currently only sits in the living room.
- a. What is your diagnosis
 - b. what are your expected findings on examination
 - c. classify/stage your diagnosis
 - d. treatment including dose, route
10. Pt with AMS, weight loss, hyperpigmentation(nausea and vomiting?) and salt craving. Urine dipstick negative for protein, red blood cells, ...
- a. Na=126 , K= 6.8 , bicarbonate =..... plus BUN and Creatinine
 - b. What is your diagnosis
 - c. what diagnostic investigations would you perform
 - d. what is the definitive management